Communication in Pediatric Nursing

Communication in Pediatric Context

Third Stage

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Pediatric Nursing

LEARNING OUTCOMES

After completing this lecture, you will be able to:

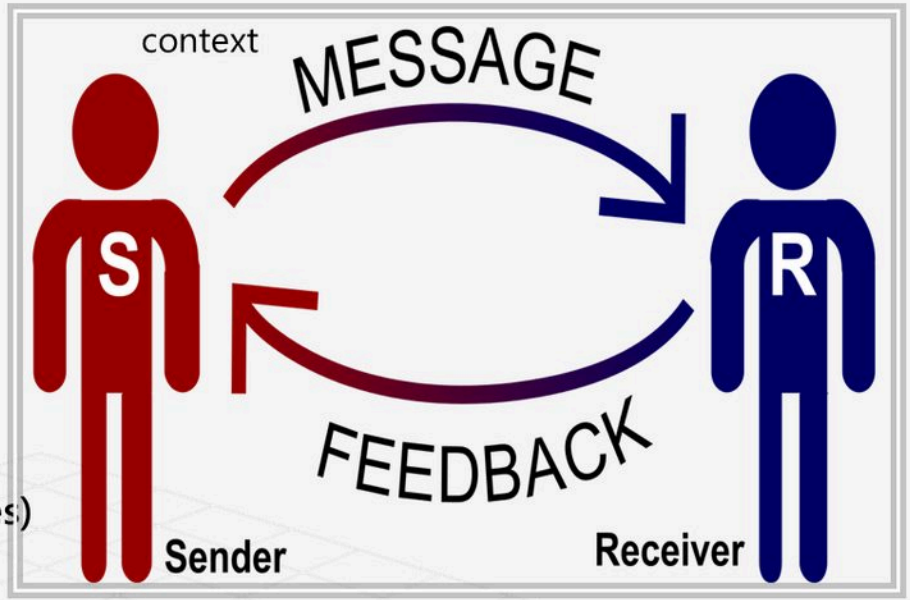
1. Define communication and explain why it is especially critical in pediatric nursing (for safety, trust, adherence).
2. Choose appropriate communication strategies when interviewing parents and children (by age).
3. Apply teach-back to pediatric families to ensure understanding.
4. Use developmentally tailored language, nonverbal cues, and distraction in pediatric procedures.
5. Reflect critically on barriers unique to pediatric settings and propose evidence-based mitigation.

Communication in Pediatric Context

- Definition: Communication is the exchange of meanings among people via a shared system of symbols (words, gestures, expressions).
- It is both **talking and listening** (i.e., sending and receiving).Effective communication requires not just sending, but verifying that the receiver understood the meaning intended.

Components of the Communication Process

- ☆ **Sender** (the nurse, or parent)
- ☆ **Message** (content, clarity)
- ☆ **Channel** (verbal, nonverbal, visual aids)
- ☆ **Receiver(s)** (child, parent)
- ☆ **Feedback** (verbal or nonverbal responses)
- ☆ **Context** (setting, developmental level, cultural background



Types of Communication: Verbal & Nonverbal

1. Verbal Communication

Verbal communication uses *words* (spoken or sometimes written). The effectiveness depends on clarity, relevance, timing, tone, pacing, and avoiding jargon.

Types of Communication: Verbal & Nonverbal

Characteristics of effective verbal communication:

- **Brief, clear, well-timed**
- **Appropriate pacing** — not too fast, not so slow that attention is lost
- **Avoid medical jargon**, especially when speaking with children or caregivers
- **Repeat key points** to reinforce essential messages
- **Check understanding** (ask for feedback)
- **Warmth, interest, empathy** conveyed in voice and phrasing
- **Avoid distractions, yes/no overuse, personal bias**
- **Don’t monopolize conversation**

Nonverbal Communication in Pediatrics

- Nonverbal communication includes everything *other than the words* used: gestures, facial expressions, posture, eye contact, touch, spatial distance, timing, and tone. Many theorists believe that a large portion of meaning is carried nonverbally.

Nonverbal Communication in Pediatrics

Reasons nonverbal is powerful:

- When verbal and nonverbal conflict, listeners tend to believe nonverbal cues.
- Nonverbal cues often convey emotions, attitudes, and relational messages more than the words themselves.
- Nonverbal is continuous, often subconscious, and cross-cultural (though interpretations vary)

Elements / dimensions of nonverbal communication:

- **Spatial relationships / proxemics** (how close you stand)
- **Appearance** (clothing, grooming)
- **Eye contact**
- **Body posture** (leaning forward, slouching, turned away)
- **Gestures** (emphasizing, pointing)
- **Facial expressions** (smile, frown, raised eyebrows)
- **Timing / hesitation / pauses**
- **Touch** (appropriate, comforting)

Scenario:

- A nurse enters a child's room to explain a procedure. The nurse sits at the child's eye level (nonverbal: reduces intimidation), uses a soft tone, and smiles gently. The nurse says, "I'm going to check your breathing for a few seconds." The child nods. The mother asks, "Will it hurt?" The nurse pauses, touches the child's shoulder (if acceptable), and answers truthfully, "You may feel a little pinch, but it will be fast, and I'll be right here with you." Because nonverbal and verbal align (gentle posture, reassurance), the child and parent are calmer.

Developmental Level Communication Principles

Infants (0–12 months)

Principles:

1. Allow the infant time to warm up to strangers.
2. Respond to the infant's cries in a timely manner.
3. Use "motherese" (infant-directed speech), a soothing and calm voice.
4. Talk to the infant directly (even though caregiver is present).

Developmental Level Communication Principles

Toddlers (1–3 years)

Principles:

1. Approach at eye level if possible.
2. Approach the toddler carefully; the child may be fearful.
3. Integrate the toddler's words for familiar objects or activities into care.
4. Prepare for procedures right before they are carried out.
5. Use dolls, storytelling, and picture books in conversations.
6. Stand at eye level (repeated to emphasize).

Developmental Level Communication Principles

Preschoolers (3–5 years)

Principles:

1. Allow choices when appropriate.
2. Use play, storytelling, puppets, and third parties.
3. Speak honestly, use simple language, and be concise.
4. Prepare for procedures 1 hour or less before they occur.

Developmental Level Communication Principles

School-Age Children (6–12 years)

Principles:

1. Use books, diagrams, and videos to prepare for procedures.
2. Prepare for procedures several days in advance.
3. Allow honest expression of feelings and adequate time for questions.

Developmental Level Communication Principles

Adolescents (13–18 years)

Principles:

1. Prepare up to 1 week prior to experiences.
2. Respect the adolescent's need for privacy.
3. Use appropriate medical terminology (but explain it).
4. Use creative methods to explain experiences and procedures.
5. Respect the adolescent's right to refuse treatment (within legal/ethical limits).

Thank you